

EXECUTIVE BRIEF

# V7\_B\_N15 - National Health Intelligence Capstone

What leaders should authorize, require, and refuse

|                 |                                                              |
|-----------------|--------------------------------------------------------------|
| Production Unit | PU-B07-C35                                                   |
| Decision status | Controlled learning asset                                    |
| Audience        | Executives, programme owners, statisticians, technical leads |
| Data            | Synthetic examples only                                      |

**Executive takeaway:** Integrate governed health evidence, surveillance, capacity, equity, evaluation, referrals, and accountability into a national decision product.

## Why it matters

Integration without governance can centralize error and weaken clinical and public accountability.

## Illustrative application

A national brief connects indicators, risks, resources, uncertainty, and authorized actions.

## Minimum approval conditions

- Named accountable owner and lawful purpose
- Defined evidence, provenance, reference period, population, and geography
- Decision-relevant validation and visible uncertainty
- Documented abstention, referral, override, and incident processes
- Accessibility, equity, security, retention, and retirement review

## Non-negotiable boundary

The analytical system may inform review. It may not independently authorize benefits, sanctions, enforcement, clinical treatment, school exclusion, displacement, or another consequential action.